# Eastside Community Health Clinic AI use policy

Draft for board review · Effective when the board approves it · Owner: Executive director, until the board names an owner · Next review: in 12 months

**Why we have this policy.** We want AI to give providers their evenings back. We must protect patients and keep care safe.

**Who it covers.** Staff, volunteers, contractors and board members, at both locations.

### TOOLS WE USE

- **EHR AI scribe:** Three-provider pilot, business associate agreement signed. May hold patient information in those visits.
- **New tools, add-ons and AI features.** Nadia (privacy and security officer) approves any new tool or AI feature that could touch patient information, after a business associate agreement. Beth (COO) approves others; clinical uses also need Dr. Okafor.
- **Accounts.** Use your work email. Turn off any vendor setting that trains on what you type.

### WHAT YOU CAN USE AI FOR

- **Everyone, now.** Any AI tool, through your work email, for work with no information about a real person.
- Appointment reminder and sliding fee letter templates, with no patient details.
- Patient education handouts, with no patient details.
- Clinicians: general clinical lookups with no patient information.
- Pilot providers: scribe-drafted visit notes.

### NEEDS APPROVAL FIRST

- Patient information in any tool beyond the scribe pilot — *Nadia, privacy and security officer*
- Any other clinical use — *Dr. Okafor, medical director*
- Billing or coding help — *Your manager*

### OFF LIMITS

- **Never put this into an AI tool** unless the tool is approved for it above: Unless a tool above is approved for it: diagnoses, medications, behavioral health notes, HIV status and reproductive health (HIPAA). Substance use treatment records, with extra federal protection. Immigration status, sliding fee income documents, insurance numbers, staff files, the donor list, and all passwords.
- No tool that could touch patient information without a business associate agreement.
- AI never makes a clinical decision or signs a note.

### CHECK EVERYTHING BEFORE IT GOES OUT

- A clinician reviews every word of AI notes and handouts before signing or sending. Check medications, names, numbers, tone.
- A person checks billing and coding suggestions. Whoever sends it is responsible.

### DECISIONS ABOUT PEOPLE

- A person, not AI, decides sliding fee eligibility and who gets services. AI may help a clinician look things up.

### BEING OPEN ABOUT AI

- Tell patients when the scribe is listening, and let them say no.
- If we add a patient chatbot, it must say it is not a person.

### IF SOMETHING GOES WRONG

- Tell Nadia right away. She runs the HIPAA breach clock.
- Reporting a mistake is expected, never punished.

### WHO OWNS THIS POLICY

- The compliance committee reviews this policy; the board approves it.

### OPEN QUESTIONS FOR THE BOARD

- After the pilot, do we roll the scribe out to all providers, and should behavioral health use it?
- Do we pay for or approve ChatGPT, OpenEvidence or UpToDate AI search for any use with patient information?
- Who checks AI or Google Translate versions of Spanish patient materials?
- What do we tell funders, like HRSA, who ask about AI?

Sample policy for an invented organization, drafted by Fulton Ring from a mock 30-minute call. Not legal advice; not reviewed by a real nonprofit.